

ORDER

Co-Prioritize Interventions



MONIQUE T. CLASS, MS, APRN-BC

APPLYING FUNCTIONAL MEDICINE IN CLINICAL PRACTICE

Disclosures

Monique T. Class, MS, APRN-BC, disclosed no relevant financial relationships with any ineligible companies.

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Learning Objectives

At the conclusion of this segment, successful participants will be able to:

1. Elicit and understand the patient's commitment, their readiness for change, their circumstances, and what matters most to them.
2. Elicit and align the patient's goals, values, purpose, and preferences with your explanation of their condition(s) to collaboratively prioritize the selection and sequence of interventions.
3. List the elements of SMART goals and explain how they help to determine the most feasible initial interventions for patients.

IFM Clinical Skills and Performance Objectives

This segment will enable participants to develop & apply these skills in their practice:

1. Elicit and understand the patient's readiness for change, their circumstances, and what matters most to them.
2. Elicit the patient's goals, values, purpose, and preferences and align these with your understanding of their physiological dysfunctions to collaboratively prioritize the selection and sequence of interventions.
3. Use the principles of SMART goals to design and implement the most feasible initial interventions for the patient.

GOTOIT Functional Medicine Framework

G

GATHER oneself and clinical indicators on the IFM Functional Medicine Timeline.

O

ORGANIZE clinical indicators and imbalances on the IFM Functional Medicine Matrix.

T

TELL the patient's story to ensure a shared understanding.

O

ORDER and prioritize interventions feasible for the patient.

I

INITIATE referrals, evaluative tests, and therapeutic modalities.

T

TRACK and adjust evaluative and therapeutic regimens.

The Functional Medicine Clinical Method

GOTOIT:

The six steps of the functional medicine clinical method

GOTOIT Functional Medicine Framework

- G** **GATHER** oneself and clinical findings on the IFM Functional Medicine Timeline.
- O** **ORGANIZE** clinical findings and imbalances on the IFM Functional Medicine Matrix.
- T** **TELL** the patient's story to ensure a shared understanding.
- O** **ORDER** and prioritize interventions feasible for the patient.
- I** **INITIATE** referrals, evaluative tests, and therapeutic modalities.
- T** **TRACK** and adjust evaluative and therapeutic regimens.

Clinical Focus

- Description and main features of the Order step of the GOTOIT method.
- Patient agency and “health as a skill set.”



Order

Order and collaboratively prioritize the most feasible and effective interventions by taking functional medicine model factors and patient contextual factors into account.

The ORDER Step of GOTOIT

ORDER AND PRIORITIZE:

- Appropriate tools to gather data on clinical imbalances, ATMs, and patient preferences
- Feasible and effective interventions

 GOTOIT <i>The sequence of steps used in the practice of functional medicine:</i>		
GOTOIT Step and Definition	IFM Tools (examples)	Clinical Actions
G – GATHER onself with a brief personal practice prior to prepare for each patient encounter. Take a detailed functional medicine history that includes the essential health events and features of a patient's life documented on the IFM Timeline in the process of organizing the patient's data.	• Cultivating Self Awareness and Mindfulness • Adult Intake Form • Medical Symptom Questionnaire (MSQ) • PROMIS Questionnaires • Diet, Nutrition, and Lifestyle Journal • Functional Medicine Tree • Functional Medicine Timeline	• Use the Cultivating Self-Awareness and Mindfulness document to help gather yourself before every patient encounter. • The Adult Intake Form is used to gather initial intake information. Whereas, the MSQ, PROMIS Questionnaires, and Diet, Nutrition, and Lifestyle Journal help gather initial symptom, function, and dietary information. • Map root causes (antecedents, triggers, mediators (ATMs)) and effects (clinical findings, current concerns) of physiological dysfunction on the Functional Medicine Timeline.
O – ORGANIZE the data gathered in the previous step and document them on the IFM Timeline and Matrix to identify the personalized root causes of current concerns, dysfunctions, and conditions, and to begin to prioritize the order (sequence) of interventions.	• Functional Medicine Tree • Functional Medicine Timeline • Functional Medicine Matrix • Adult Intake Form • Medical Symptom Questionnaire (MSQ) • PROMIS Questionnaires • Diet, Nutrition, and Lifestyle Journal	• Using the Functional Medicine Matrix, populate ATMs, clinical findings, and current concerns from the Timeline. Add modifiable lifestyle factors (MLFs) and mental, emotional, spiritual factors (MES) to support the identification of the most important areas of physiological dysfunction.
T – TELL the life story back to the patient to ensure accuracy and a shared understanding. This dialogue, focused on the Timeline, highlights how factors elicited in the Gather step may have contributed to current or future clinical findings, concerns, health status and trajectory. The patient is asked to join in correcting the story, engendering a context of true partnership.	• Functional Medicine Matrix • Functional Medicine Timeline • Functional Medicine Tree	• The Functional Medicine Timeline is used to re-tell the patient's story while correlating prior events and root causes with current concerns or conditions. Ask the patient to join in for corrections or additional context. • Use the Functional Medicine Tree to explain how the root causes have contributed to the patient's condition.

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Long Description: The ORDER Step of GOTOIT

Tool kit item: G O T O I T

Steps practicing functional medicine

This item is available in the tool kit

What Determines Health?

Promoting Patient Agency

- “Health is determined by many factors.”
- “Health does not rely on luck or fate.”
- “Health is a skill set, and you can learn these skills.”
- **“Mindset about health is pivotal for success.”**

What Determines Health?

Promoting Patient Agency

- “Your strengths can help you learn these skills.”
- “What strengths have helped you overcome previous life challenges or health challenges?”
- *Continue to leverage the patient’s answers to this question to motivate and to develop the therapeutic partnership.*

The Patient Must Be Self-Motivated

Establish Level of Commitment

How interested are you in using your strengths to learn new health skills and start feeling better?

**“Skills not
pills”**

**“A skill for
every ill”**

Clinical Focus

- Role of the patient's goals, values, preferences, and priorities.
- Therapeutic partnership and what matters to the patient.



High-Yield Best Practices

ORDER AND PRIORITIZE

- Ask the patient what they believe are the greatest contributors to their current concerns.
- Elicit the patient's goals, values, preferences, and priorities.
- Understand the patient's circumstances and capacity to change.
- Begin with a plan that is feasible for the patient in their context.
- Begin with a plan that prioritizes prompt relief of symptoms.

High-Yield Best Practices

ORDER AND PRIORITIZE



Establish the habit before you optimize the habit.



Start with the first small step and build.



The first step is determined by the patient, not the clinician.

**What
questions do
you ask...**

**...and how do
you ask them?**

Whole Person Driven Care

Patient Goals, Values, Preferences, and Priorities

What? – Why? – When? – How?

- **What?** What are the patient's health goal(s)?
- **Why?** What matters to the patient?
- **When?** When will the patient start, and for how long?
- **How?** What initial change/skill/step is most feasible?

Whole Person Driven Care

Patient Goal(s)

What? – Why? – When? – How?

- What? Health goal
- How would you complete this sentence?
 - “My health goal(s) are to _____.”

Whole Person Driven Care

Patient's Priorities: What? – Why? – When? – How?

Why?

- Meaning and purpose – what matters to the patient?

How would you complete this sentence?

- “I want to be healthier because _____.”
- “I want my health for _____.”
- “If I was healthier, I would _____.”

The Whole Health Approach

What's the matter
with the patient?

What matters
to the patient?

Whole Person Driven Care

Patient Preferences

What? – Why? – When? – How?

- When? Initiation and duration
- How would you complete this sentence?
 - “I am willing to start using new health skills _____.” (When?)
 - “I am willing to devote _____.” (How much time?)

Whole Person Driven Care Patient Preferences What? – Why? – When? – How?

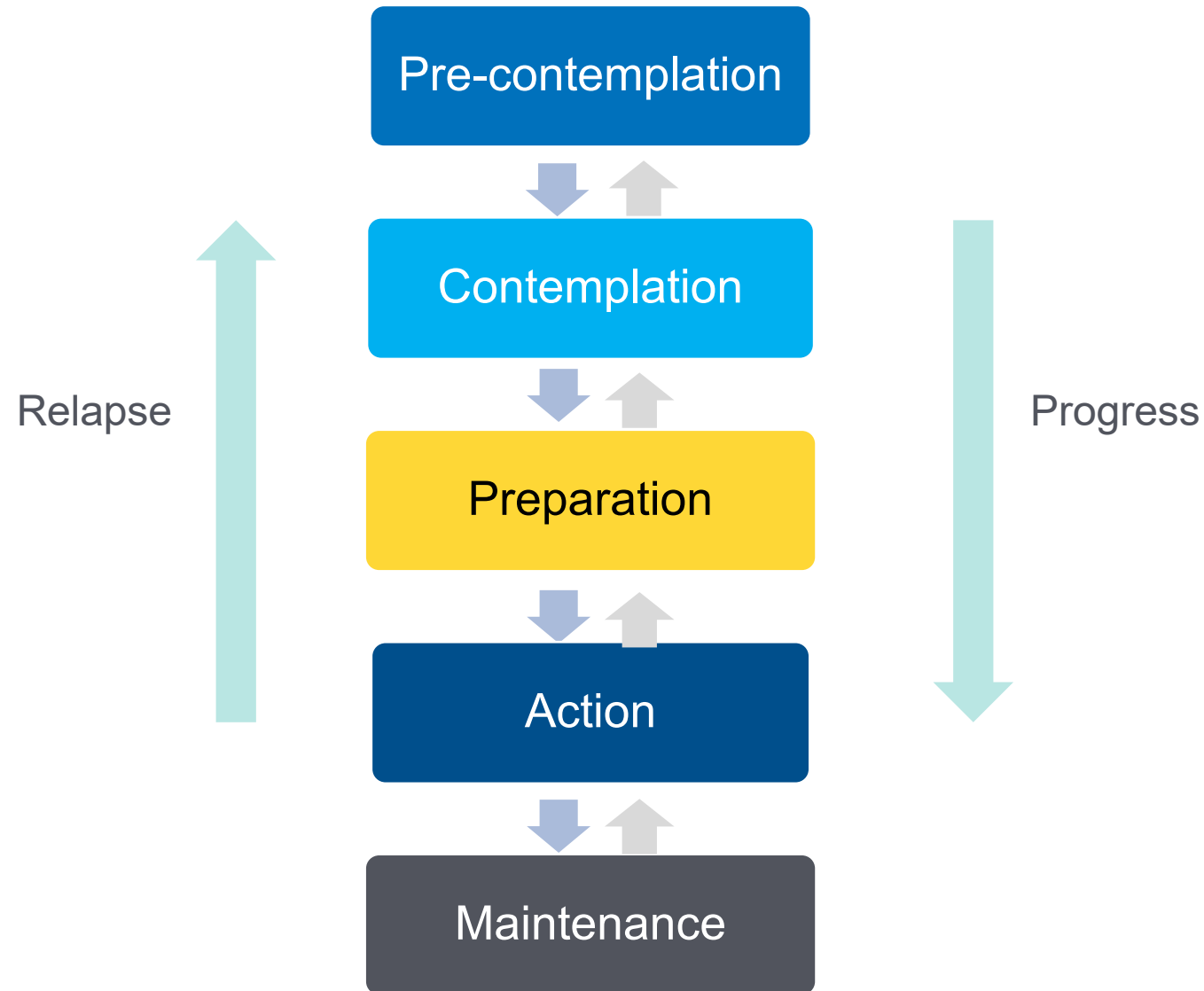
- How? See next “Clinical Focus” section.

Clinical Focus

- Stages of change and prioritizing interventions



Stages of Change

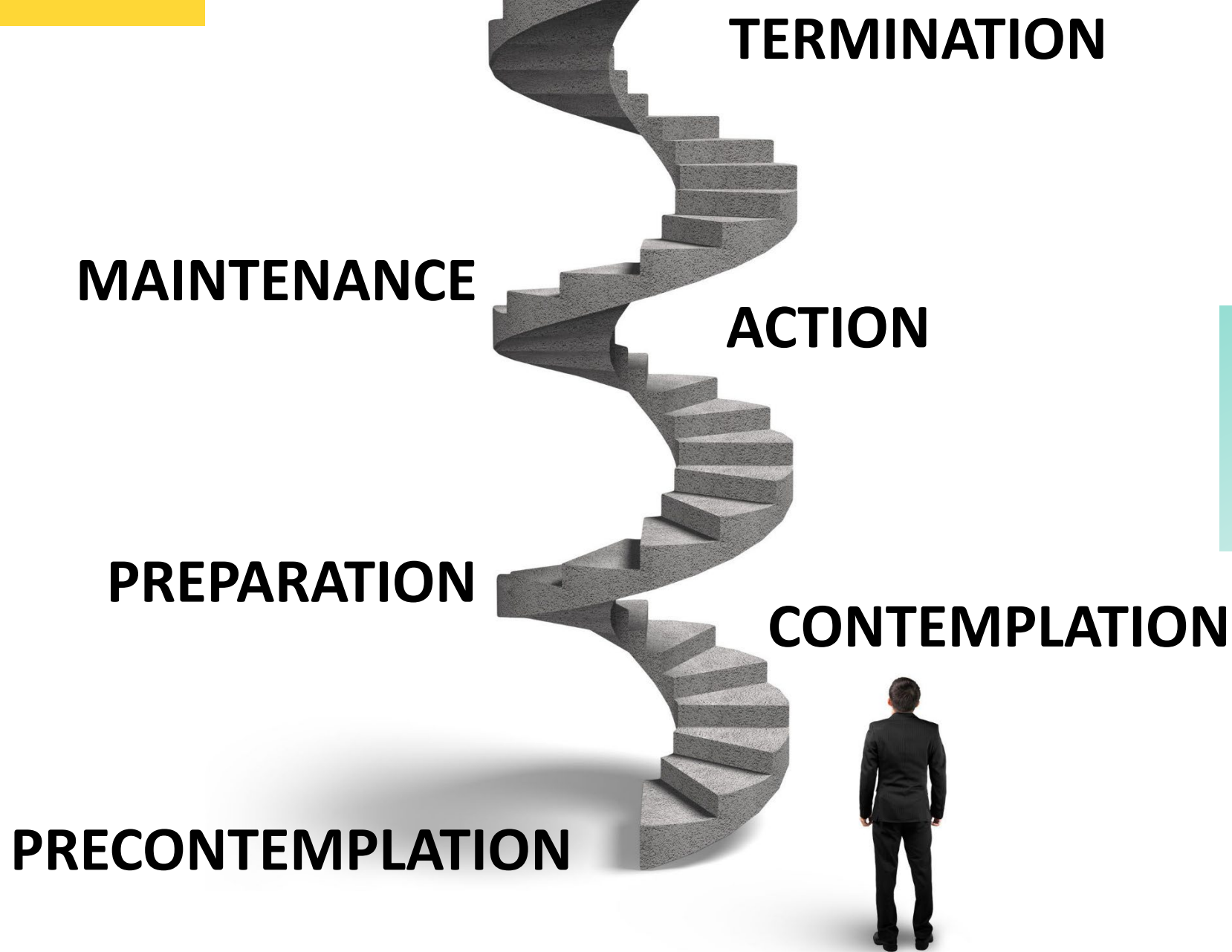


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Long Description: The Stages of Change

The stages of change include pre-contemplation, contemplation, action, and maintenance. People can progress through each stage or relapse into a previous stage.

The Spiral of Change



- Mizuno A, Kaneko H, Suzuki Y, et al. Enduring relevance of the stages of change model for transforming lifestyle behaviors. *Circ J.* 2023;87(8):1138-1142. doi:10.1253/circj.CJ-23-0292
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Long Description: The Spiral of Change

The spiral of change depicts the spiral staircase a person climbs as they progress through the stages of change. From the bottom of the staircase to the top is precontemplation, contemplation, preparation, action, maintenance, and termination.

Cycle of Change

- Most individuals find themselves “recycling” through the stages of change several times (“relapsing”) before change becomes truly established.
- **Only 5%** of self-changers followed a linear route from pre-contemplation to termination without at least one setback.

Assessing a Patient's Stage

Maintenance

I solved my problem more than six months ago.

Action

I have taken action on my problem within the past six months

Preparation

I am intending to take action in the next month.

Contemplation

I am considering taking action in the next six months.

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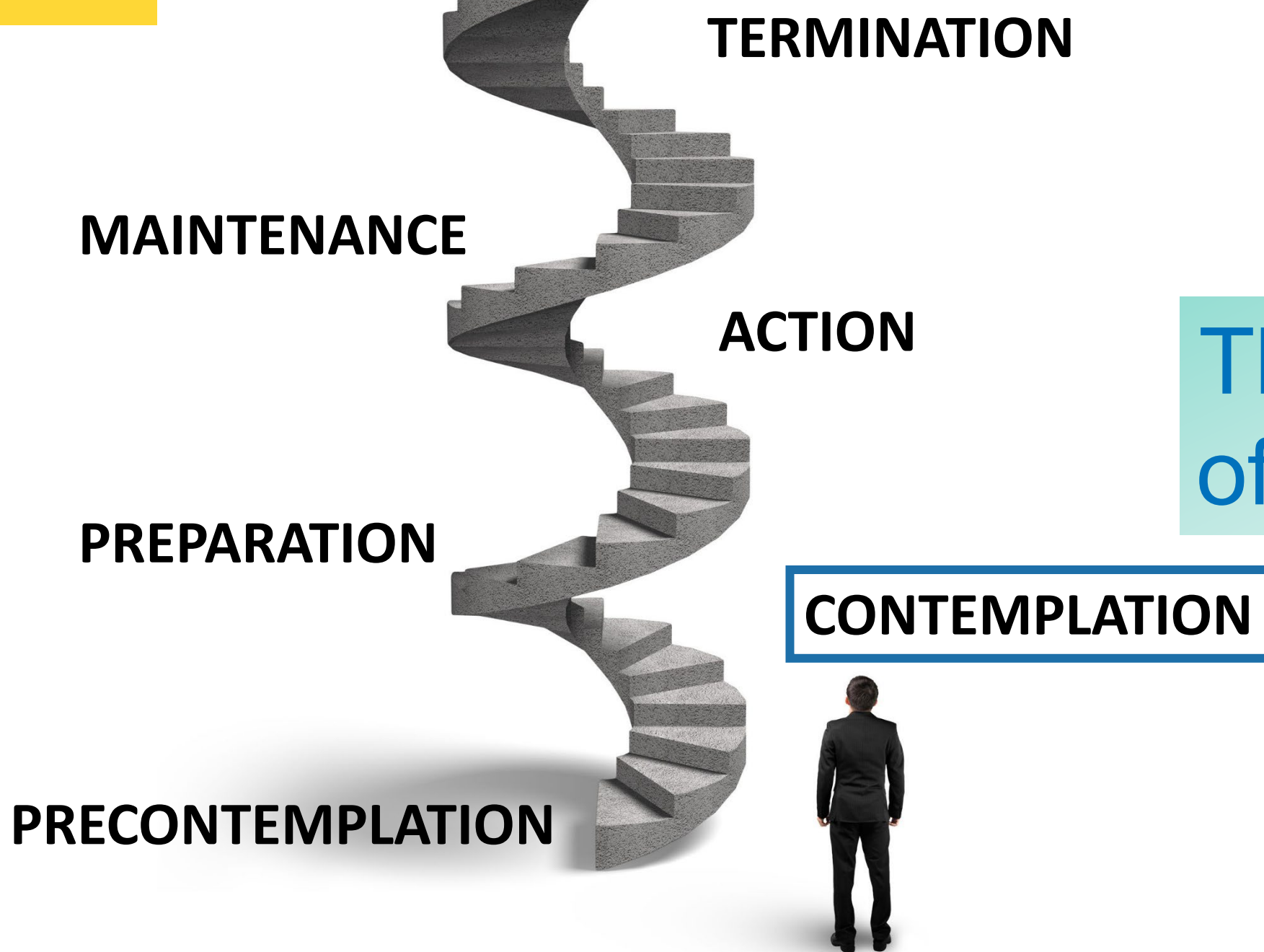
What's Precontemplation?

- Lack of intention to change behavior
- Denial of problem
- What does it look like?

•

Everyone thinks you have a problem, but ***you*** don't think you have a problem...therein lies the problem.

The Spiral of Change



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Contemplation

Chronic contemplators substitute thinking for action:

- Searching for absolute certainty
- **Waiting for the magic moment...**after the divorce is final
- Wishful thinking
- Action now is premature

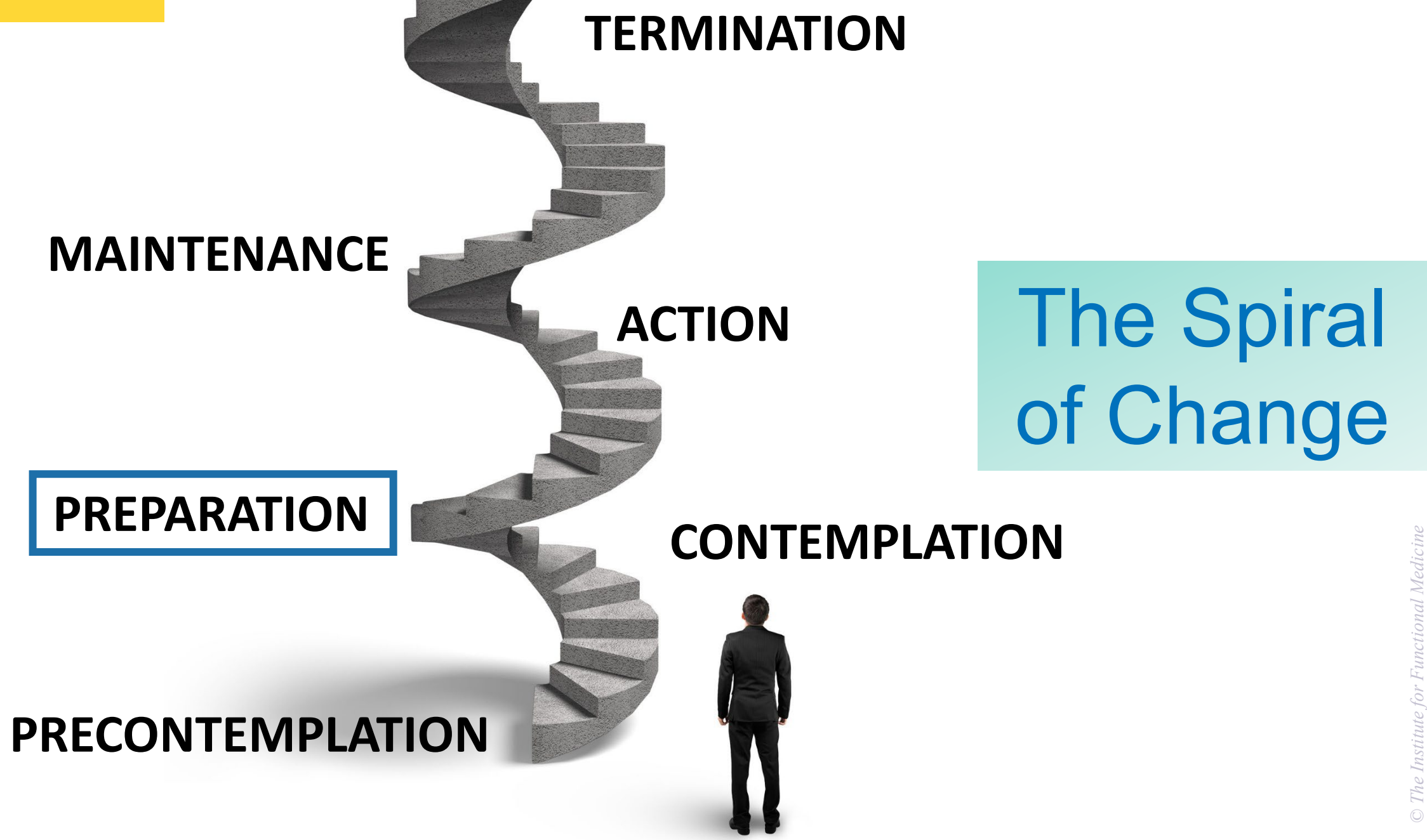


What Contemplators Do

1. Psychodrama
2. Grieve loss
3. Experiencing and expressing feelings about one's problems and solutions

Helping People Move From Contemplation:

- Show why the moment for change is now
- Help identify **EASY** action steps
- Agreement that the next step is to come up with a plan, with an agreement, for an action



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Preparation: Commitment to Action



Setting a date for action:
Publicly announcing the intended change.
A plan of action with strategies and dates.



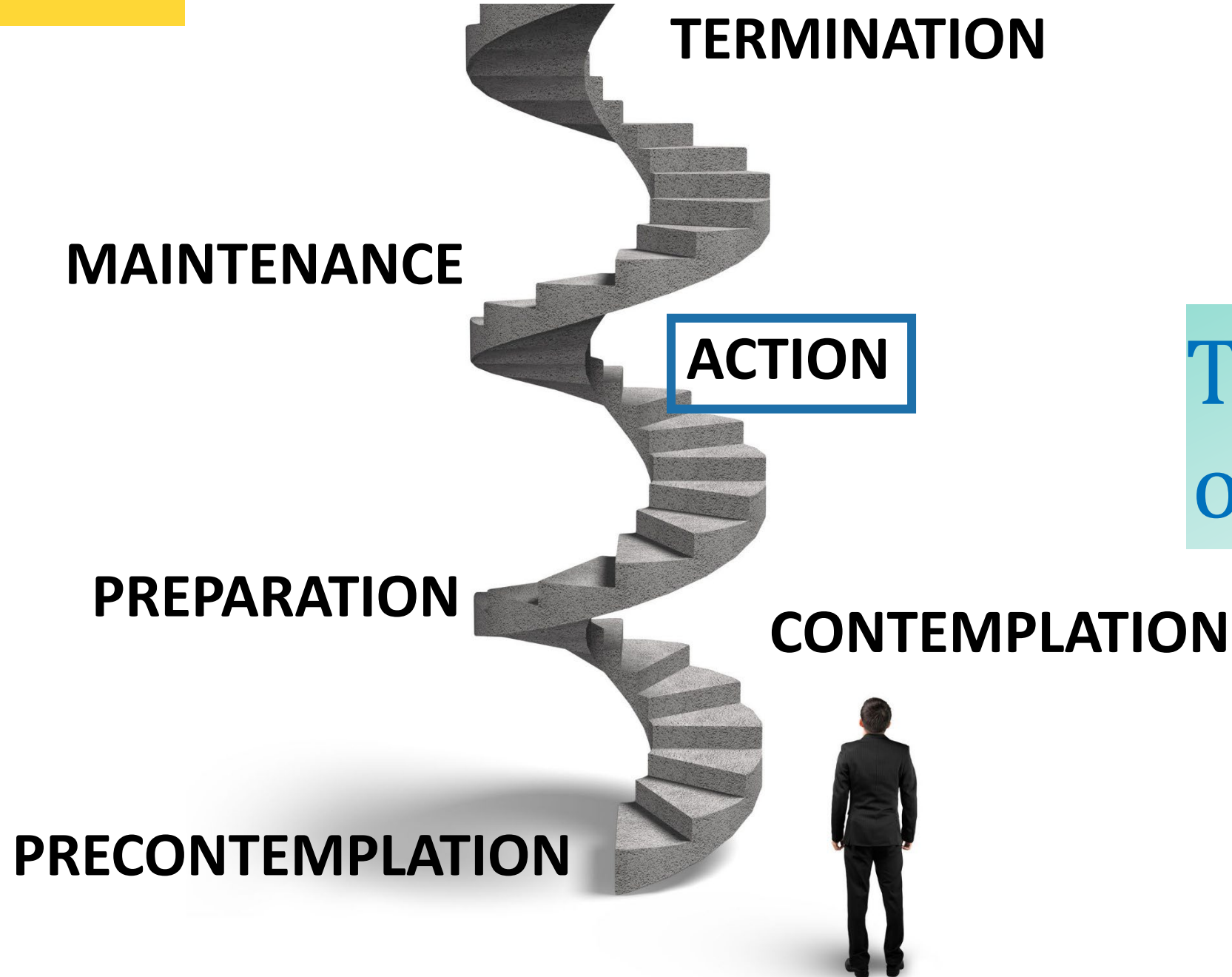
Commitment to action

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How to Help Patients Move From Preparation:

- Coach to instill belief in **ability to take action**
- Small, easily obtainable goals:
 - Buy running shoes
 - Get the gym membership

ACCOUNTABILITY AND AGREEMENT!



Long Description: The Spiral of Change

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Action



Overtly modify behavior and surroundings.

Stopping the behavior.

Confronting their fears.



They make the move for which they have been preparing.

Assisting Action

Reward:

- Rewarding self, or being rewarded by others, for making changes

Environmental Control:

- Avoiding stimuli that elicit problem behaviors
- Environmental restructuring (e.g., removing alcohol or fattening foods), avoiding high-risk cues (e.g., going out with friends that you know are not supportive of your changes)

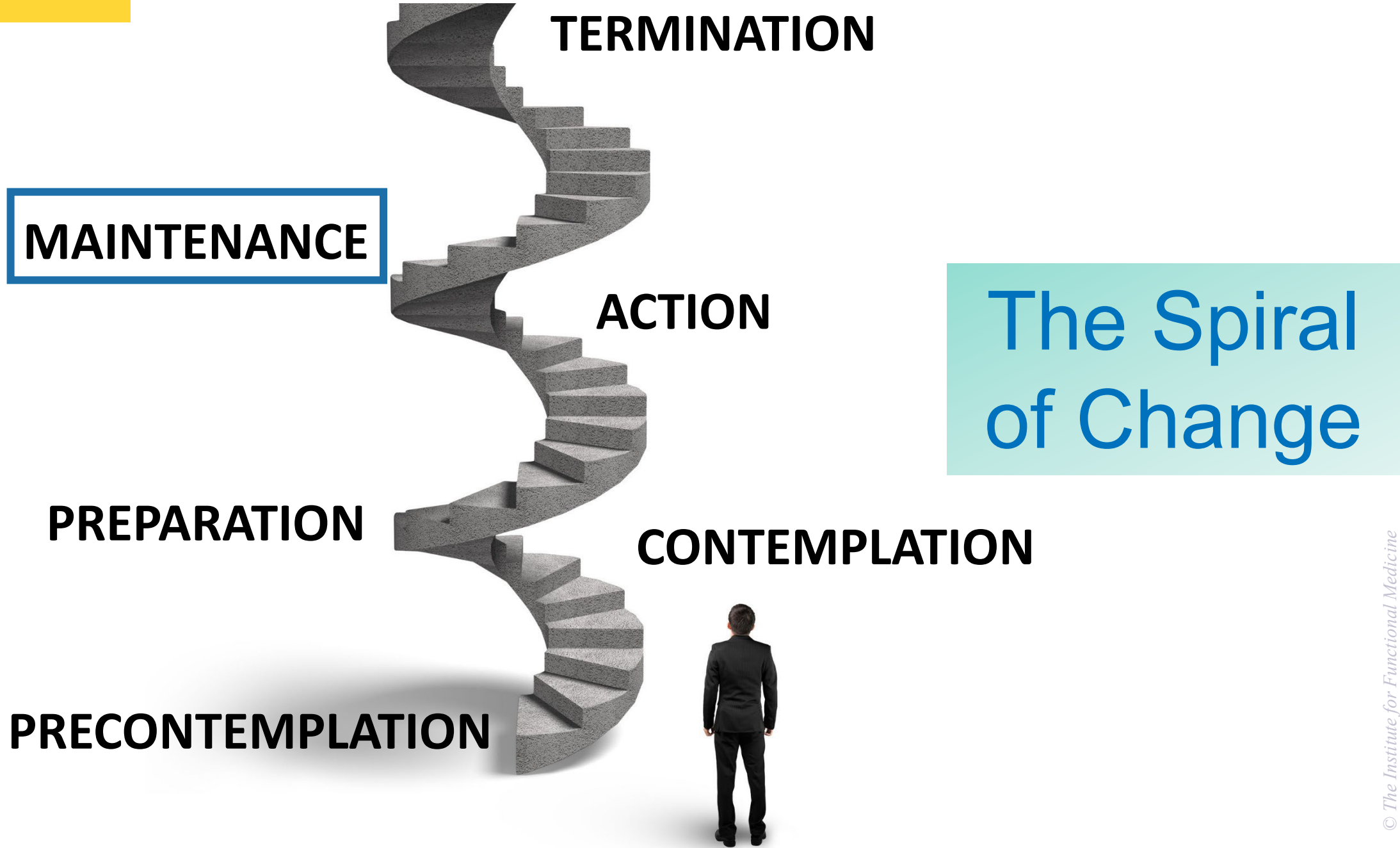
Assisting Action

Encourage Supportive Relationships:

- Enlisting the help of someone who cares
- Group support (detox groups with daily emails)

Countering – Substituting Alternatives (Active Diversions) for Problem Behaviors:

- Relaxation, affirmations, exploring spirituality, spending time with (or away from) your partner, talking to your children, exercising, visiting a friend, calling a sponsor, hobbies, reading, and even household chores.



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The spiral of change staircase with an emphasis on maintenance. The spiral of change depicts the spiral staircase a person climbs as they progress through the stages of change. From the bottom of the staircase to the top is precontemplation, contemplation, preparation, action, maintenance, and termination.

Tools for Maintenance

- Commitment
- Reward
- Environmental control
- Supportive relationships
- Frequent visits (monthly) to keep momentum
If they fall off...why?

Tools for Recycling

- Reinforce it happens 95% of the time
- Need to normalize it for people...this happens on the path to change
- Remove shame and sadness
- Gather again...focus on the goal

Transformation

- When the addiction or temptation no longer exists, people exit the cycle of change.
- Some problems require a **lifetime of maintenance** and oscillation among the stages.



TERMINATION

MAINTENANCE

ACTION

The Spiral of Change

PREPARATION

CONTEMPLATION

PRECONTEMPLATION



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Long Description: The Spiral of Change

The spiral of change staircase with an emphasis on termination. The spiral of change depicts the spiral staircase a person climbs as they progress through the stages of change. From the bottom of the staircase to the top is precontemplation, contemplation, preparation, action, maintenance, and termination.

Clinical Pearls

- Change never ends with the action stage.
- Developing an active and intelligent maintenance program is critical.
- This stage can last from six months to a lifetime.
- 95% will go backwards (recycle).

Clinical Focus

- Use of SMART goals to initiate feasible interventions.



SMART Goals

Specific (simple, sensible, significant)

Measurable (meaningful, motivating)

Achievable (agreed, attainable)

Relevant (reasonable, realistic and resourced, results-based)

Time bound (time-based, time limited, timely, time-sensitive)

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Goal Setting for Behavior Change

Changing habits to consciously improve your health is no small undertaking, and making the decision to change is just the first step. Actively thinking about and planning for change will help prepare you for the process and motivate you to stay on track.

When you're ready to make a change, it is often helpful to set tangible goals. These goals can be short-term (daily, weekly, monthly) or long-term (6 to 12 months). When goal setting for behavior change, it's also helpful to set goals that are SMART: specific, measurable, attainable, realistic, and timely.

SMART Goal Components

Specific

- State the desired outcome as explicitly as possible, and target a specific area for improvement. This is the "who, what, where, when, which, and why" of your goal.
- Example: I will ride my bike to work Monday, Wednesday, and Friday each week.

Measurable

- Identify the ways in which you will track your progress, and be as specific as possible. This is the "how" of your goal.
- Example: I will walk at least five days per week for 20 minutes in the evenings to lower my cholesterol.

Attainable

- Start with small, achievable goals that are easily outlined into specific steps that will enable you to complete the goal. Then, as you meet those smaller goals, work up to intermediate goals and goals that are more difficult to achieve.
- Example: I will walk for 15 minutes three days a week at lunch, increasing my time each week by 5 minutes until I reach 30 minutes per day. I will add an extra day every two to four weeks until I reach 30 minutes of walking five days a week.

Realistic

- Create a goal that you are both willing and able to accomplish.
- Example: I will stretch for 5 minutes before bedtime, followed by 5 minutes of breath work, which will help me fall asleep faster each night.

Timely

- Set a deadline or time frame for achieving your goal to help keep you motivated.
- Example: I will run a 5K by September 15th.

To begin setting your own SMART goals, flip the page and fill in the information in the template provided.

Long Description: Goal Setting for Behavior Change

Goal Setting for Behavior Change

This item is available in the toolkit.

SMART Goal Setting for Behavior Change

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S M A R T Goal Setting for Behavior Change

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SMART Goal Components (continued)

Attainable

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S M A R T Goal Setting for Behavior Change

This item is available in the toolkit.

SMART Goals: Stress

Specific (simple, sensible, significant)

“I will start a relaxation practice to manage stress.”

Measurable (meaningful, motivating)

“I will focus on what I am grateful for.”

Achievable (agreed, attainable)

“I can commit to 10 minutes, 3 days a week.”

Relevant (reasonable, realistic and resourced, results-based)

“I need to do this in a quiet place outside my home.”

Time bound (time-based, time limited, timely, time-sensitive)

“I will start on Monday and end at our next appt.”

Clinical Focus

- IFM Toolkit items for co-prioritizing interventions in the ORDER step.



Assess the Patient's "Stages of Change"

Applying the Stages of Change

It is invaluable for both patients and practitioners to understand where an individual is within the Stages of Change model. Understanding a patient's story includes understanding their readiness for change. By identifying where a patient is within this model, supportive strategies can be used to increase confidence and support lasting behavior change.

There is no right or wrong within the Stages of Change. Each person will progress based on their readiness and the magnitude of the behavior they wish to change.



Readiness for Change

There is a process that everyone goes through when creating healthier habits. It is often referred to as the "Stages of Change" or "readiness for change." Understanding this process can aid behavior change.

In 1979, psychologist James Prochaska first described this behavior change process for those quitting smoking. Since then, the Stages of Change model that Prochaska and his colleagues developed has been used to help individuals improve eating habits, increase physical activity, manage stress, address problematic alcohol or drug use, and many other behaviors.

The Stages of Change

The Stages of Change include five steps, as described here:

1. Precontemplation Stage

People in this stage are often unaware that a behavior needs changing. If they are aware that a behavior needs to change, they do not intend to change right now.

Strategies for this stage:

- Affirm that you are on your own timeline and are not ready to make a change right now.
- Gather more information about the behavior you want to create or how your current behavior may be impacting you.
- Explore core values.

2. Contemplation Stage

People in this stage are aware that a behavior needs to change and are seriously thinking about changing it. Individuals in this stage are not ready to take action at this time.

Strategies for this stage:

- Affirm that you are on your own timeline and are considering making a change.
- Create a list of pros and cons.
- Imagine who you will be when you make this change and why this is important for you.

Long Description: Assess the Patient's Stages of Change

Readiness for Change

This item is available in the toolkit.

Visualizing Personal Health Goals

Set and Visualize Goals

If you choose to set goals for your health, visualization may be useful to help you reach them. Visualization is a subset of guided imagery. For instance, if you want to lose weight, visualizing yourself exercising—seeing yourself in exercise clothes, picturing where you exercise, and imagining the feel of the movements and your breathing—has been shown to assist with follow-through. In essence, visualizing yourself performing various activities primes your mind to actually do them.

Consider your health goals and imagine the steps you need to achieve those goals. What visualizations will work for you?



Integrating Guided Imagery and Visualization to Achieve Personal Health Goals

Guided imagery and visualization are mind-body therapies that can help improve health and well-being. These imagery techniques focus on using multiple senses (sight, sound, touch, smell) to make imagined scenarios feel as realistic as possible. Imagining specific scenes or sequences of events may help with achieving health goals.

Understanding guided imagery and visualization

Guided imagery involves visualizing scenarios or sequences to reduce stress, inspire change, and promote mental and emotional well-being. It may be led by a live person, such as a therapist, or an audio recording previously recorded by someone trained in guided imagery. Guided imagery is immersive. It focuses on imagining specific scenes in detail and incorporates as many senses (sight, sound, taste, smell, and touch) as possible.¹

Visualization is related to guided imagery, though it differs in several ways. It involves picturing something in your mind, such as a health goal you want to achieve. Like guided imagery, it may focus on imagining scenes in as much detail as possible. However, you can also use visualization to imagine performing a series of steps to reach a goal. It can be either guided or self-directed. Self-directed visualization allows you to choose what scenes to imagine or what steps to take to help reach your goals, without input from external audio suggestions. While visualization primarily focuses on visuals, you can also incorporate other senses.²

Benefits of guided imagery and visualization

Imagery techniques, like guided imagery and visualization, can help promote change by increasing preparation and motivation toward a task, behavior, or health goal.^{3,4}

Guided imagery and visualization are beneficial for relaxation^{4,5} and stress relief,^{4,7} which can have many health benefits. These techniques also have positive effects on many other lifestyle factors that contribute to overall health. Some of these lifestyle factors are: sleep,^{8,9} food consumption^{4,10} and cravings,^{4,10,11} physical activity,^{4,12} emotional regulation,^{4,13,15} self-forgiveness,^{4,14,17} and even alcohol reduction^{4,18} or smoking cessation.^{4,19}

Long Description: Visualizing Personal Health Goals

Integrating Guided Imagery and Visualization to Achieve Personal Health Goals

This item is available in the toolkit.

SMART Goal Setting for Behavior Change

SMART Goal Components

Specific

- State the desired outcome as explicitly as possible, and target a specific area for improvement. This is the “who, what, where, when, which, and why” of your goal.
- Example: I will ride my bike to work Monday, Wednesday, and Friday each week.

Measurable

- Identify the ways in which you will track your progress, and be as specific as possible. This is the “how” of your goal.
- Example: I will walk at least five days per week for 20 minutes in the evenings to lower my cholesterol.

Attainable

- Start with small, achievable goals that are easily outlined into specific steps that will enable you to complete the goal. Then, as you meet those smaller goals, work up to intermediate goals and goals that are more difficult to achieve.
- Example: I will walk for 15 minutes three days a week at lunch, increasing my time each week by 5 minutes until I reach 30 minutes per day. I will add an extra day every two to four weeks until I reach 30 minutes of walking five days a week.

Realistic

- Create a goal that you are both willing and able to accomplish.
- Example: I will stretch for 5 minutes before bedtime, followed by 5 minutes of breath work, which will help me fall asleep faster each night.

Timely

- Set a deadline or time frame for achieving your goal to help keep you motivated.
- Example: I will run a 5K by September 15th.

Accessible version in your toolkit.



Goal Setting for Behavior Change

Changing habits to consciously improve your health is no small undertaking, and making the decision to change is just the first step. Actively thinking about and planning for change will help prepare you for the process and motivate you to stay on track.

When you're ready to make a change, it is often helpful to set tangible goals. These goals can be short-term (daily, weekly, monthly) or long-term (6 to 12 months). When goal setting for behavior change, it's also helpful to set goals that are SMART: specific, measurable, attainable, realistic, and timely.

SMART Goal Components

Specific

- State the desired outcome as explicitly as possible, and target a specific area for improvement. This is the “who, what, where, when, which, and why” of your goal.
- Example: I will ride my bike to work Monday, Wednesday, and Friday each week.

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- Start with small, achievable goals that are easily outlined into specific steps that will enable you to complete the goal. Then, as you meet those smaller goals, work up to intermediate goals and goals that are more difficult to achieve.
- Example: I will walk for 15 minutes three days a week at lunch, increasing my time each week by 5 minutes until I reach 30 minutes per day. I will add an extra day every two to four weeks until I reach 30 minutes of walking five days a week.

Realistic

- Create a goal that you are both willing and able to accomplish.
- Example: I will stretch for 5 minutes before bedtime, followed by 5 minutes of breath work, which will help me fall asleep faster each night.

Timely

- Set a deadline or time frame for achieving your goal to help keep you motivated.
- Example: I will run a 5K by September 15th.

To begin setting your own SMART goals, flip the page and fill in the information in the template provided.

Long Description: SMART Goal Setting for Behavior Change

- Goal Setting for Behavior Change.
- This item is available in the toolkit.
- S M A R T Goal Components
- Specific: state the desired outcome as explicitly as possible, and target a specific area for improvement. This is the “who, what, where, when, which, and why” of your goal.
- Example: I will ride my bike to work Monday, Wednesday, and Friday each week.
- Measurable: Identify the ways in which you will track your progress, and be as specific as possible. This is the “how” of your goal.
- Example: I will walk at least five days per week for 20 minutes in the evenings to lower my cholesterol.
- Attainable: start with small, achievable goals that are easily outlined into specific steps that will enable you to complete the goal. Then, as you meet those smaller goals, work up to intermediate goals and goals that are more difficult to achieve.
- Example: I will walk for 15 minutes three days a week at lunch, increasing my time each week by 5 minutes until I reach 30 minutes per day. I will add an extra day every two to four weeks until I reach 30 minutes of walking five days a week.
- Realistic: Create a goal that you are both willing and able to accomplish.
- Example: I will stretch for 5 minutes before bedtime, followed by 5 minutes of breath work, which will help me fall asleep faster each night.
- Timely: Set a deadline or time frame for achieving your goal to help keep you motivated.
- Example: I will run a 5 K by September 15th.

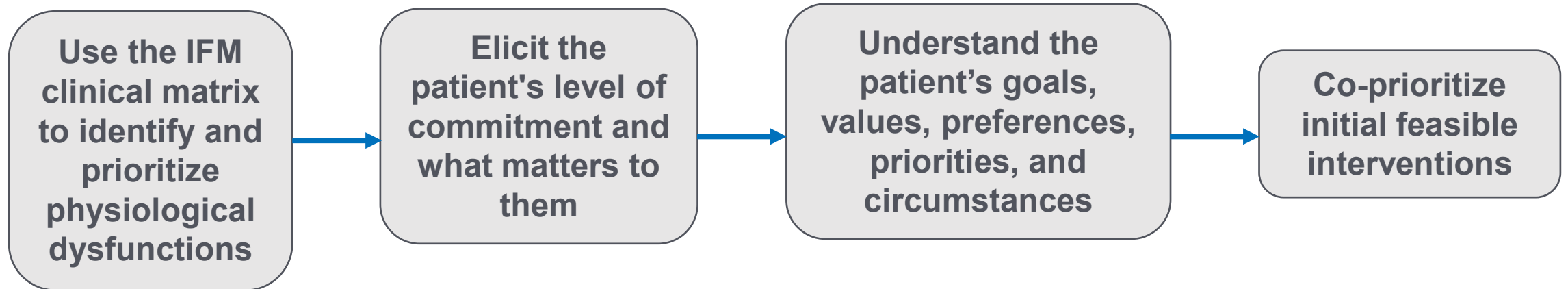
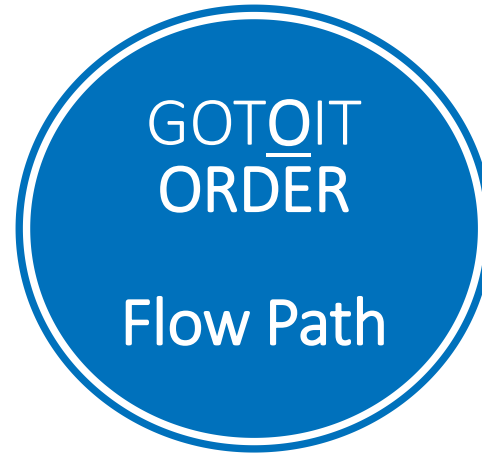
Clinical Focus

- Summary, conclusion, and key takeaways.
- Clinical flow path.



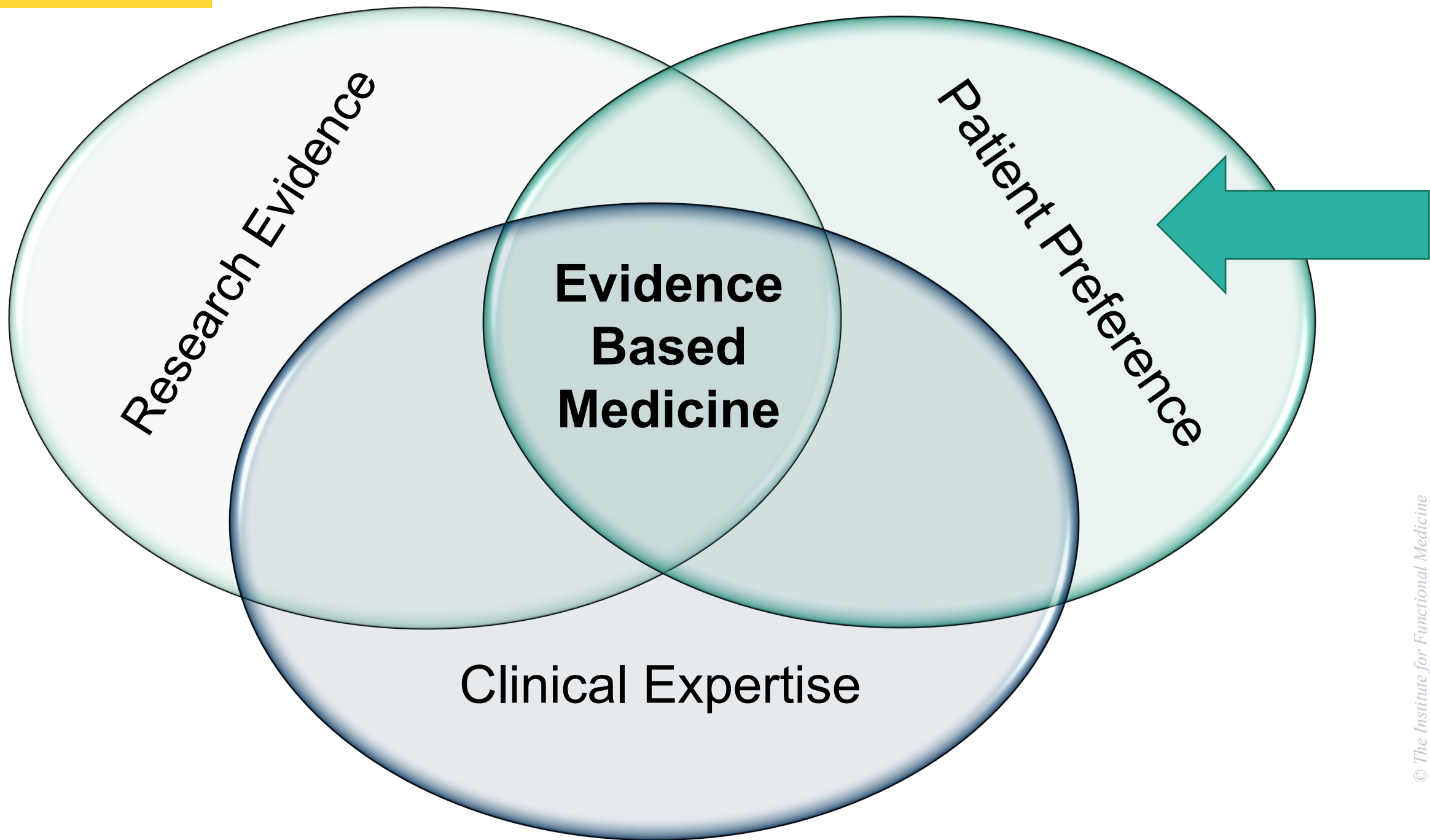
Key Clinical Takeaway:

Flow Path for the Order step of GOTOIT



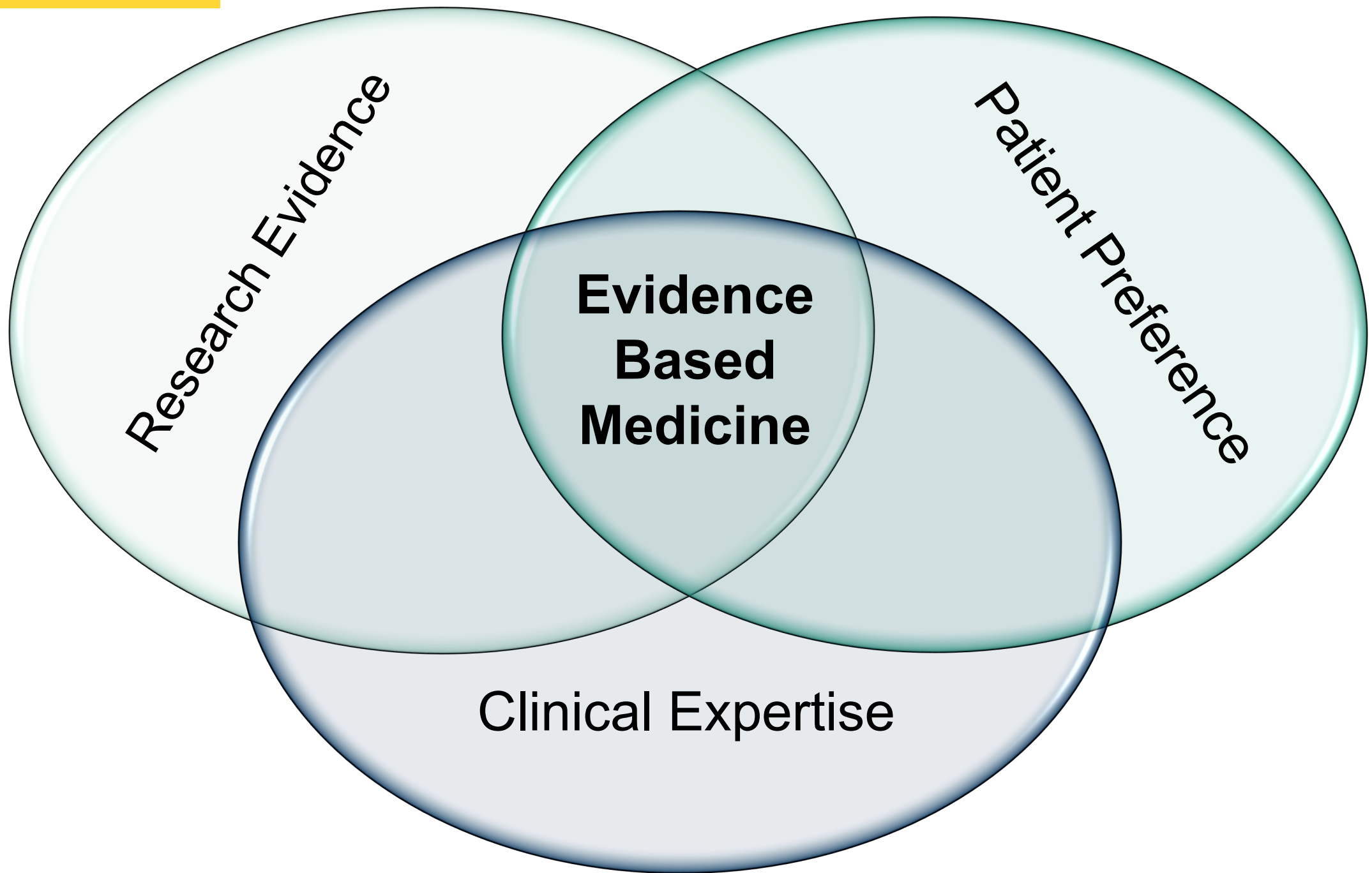
Long Description: The GOTOIT ORDER

- The G O T O I T O R D E R Flow Path
- Start by using the matrix to identify and prioritize psychological dysfunctions
- Next, elicit the patient's level of commitment and what matters to them
- Then, understand the patient's goals, values, preferences, priorities, and circumstances
- Finally, co-prioritize initial feasible interventions



Long Description: No title on slide

This image depicts a Venn Diagram of three overlapping circles titled Research Evidence, Patient Preferences, and Clinical Expertise. Evidence Based Medicine overlies where these circles meet.



Long Description: No title on slide

This image depicts a Venn Diagram of three overlapping circles titled Research Evidence, Patient Preferences, and Clinical Expertise. Evidence Based Medicine overlies where these circles meet.

**What's the
matter with
you?**

**What matters
to you?**

What's *wrong*
with you?

What's *strong*
with you?